Access Denied: Hospital Closures and Emergency Care Deserts in New York State

Leslie Reynolds

Cornell University Program on Applied Demographics

Cornell Brooks Public Policy

2025 budget reconciliation bill (PL 119-21)

Formerly known as the One Big Beautiful Bill Act (OBBBA)

Became public law in July 2025

- Implements new restrictions and cuts to federal programs
- Expected increase in uninsured population in 2026 by 1.3 million (Congressional Budget Office, 2025)

Cuts the Medicaid provider tax “safe harbor” threshold from 6% to 3.5% by 2031 for states that expanded Medicaid eligibility under the ACA

- Medicaid pays lower fees for care than Medicare or private insurance- provider taxes help pay facilities back for services to Medicaid patients

Rural hospitals rely more heavily on payments from Medicaid taxes due to smaller profit margins

- $50 billion Rural Health Transformation Program (RHTP) created to try and counteract this impact, but differences in effectiveness expected between states

New Report on State Impacts of PL 119-21

"State-Level Impacts of Key Medicaid Provisions in the One Big Beautiful Bill Act" (Rao et al., 2026) laid out the 11 new Medicaid provisions and analyzed the impacts across states

- New York State's Medicaid fund expected to decline 2.1% by 2028 (about $8.4 billion)
- Estimated to be most detrimental for NY state's Medicaid fund (by 2028):
  - **New work Requirements**- estimated loss of $3.8 billion
  - **Limits on provider Taxes-** estimated loss of $2.9 billion
  - **Eligibility redeterminations** (in expansion states) every 6 months- estimated loss of $2.1 billion
- All states will receive between $100 million and $300 million annually from the Rural Health Transformation Fund (RHTF)
  - States with smaller Medicaid populations and lower Medicaid spending gain more as a proportion of the total Medicaid budget
  - Estimated that Kentucky will receive the most at $226 million; Rhode Island the least at $156 million

Impacts on Hospitals

- Hospital closures already a persistent problem
  - 63 hospitals closed in New York State since 2000 (NYS Department of Health, 2026)
- “Emergency Care Deserts”: living 30 minutes or more from a hospital that provides emergency care
  - Already common especially in rural areas
- Brief from the Fiscal Policy Institute (FPI) listed hospitals in NYS at risk of closure under cuts
  - Hospitals receiving > 25% of net patient revenue (NPR) from Medicaid and government appropriations would be in immediate risk of closure (Eisner & Kinnucan, 2025)
  - Hospitals with between 20% and 25% of NPR from Medicaid/govt. appropriations could also be at risk of closure
- FPI listed 70 hospitals in NYS most at risk of closure under these definitions
  - 31 additional NYS hospitals at risk (> 20% and < 25%)

Data & Methods

- New York State DOH Facility Data (updated 2025) on hospital locations and type
- Hospital location data from neighboring state Health Departments (Pennsylvania, New Jersey, Connecticut, Massachusetts, and Vermont)
- 2024 CMS data from National Academy for State Health Policy's (NASHP) Hospital Cost Tool (HCT)

<u>Building the Hospital Database:</u>

1. Merged state hospital location data to NASHP/CMS data
    a. By address then by name, due to variation in naming (between sources, across years, etc.)
    b. If name was close but had a different address > verified facility type and location
    c. If facility was in one list but not the other- verified it was still a hospital, still open, and had an emergency room, and added or removed if necessary

2. Imported into QGIS and validated location data
    a. Geocoded addresses when needed- some had mailing addresses (such as a P.O. Box) rather than a physical address, or incorrect address information

Data & Methods (cont.)

Calculated share of Net Patient Revenue (NPR) from Medicaid and government appropriations

- “High Risk” if 25% or more of hospital NPR from Medicaid/ govt. appropriations
- “Potential Risk” if between 20 and 25% of hospital’s NPR was from Medicaid/ govt. appropriations

Dealt with Missing Data

- Hospital was in NYS and missing 2024 Medicaid data = imputed risk value from the FPI list (which used 2023 data); If not listed, used 2023 CMS data.
- Hospital not in NYS and missing 2024 Medicaid data = substituted value from 2023 CMS data, given hospital was still open.

Defining Emergency Care Deserts (ECD):

> 5 miles from the closest hospital in the 5 NYC boroughs (Bronx, Brooklyn, Manhattan, Queens, or Staten Island) or neighboring counties of Nassau, Rockland, or Westchester

> 15 miles from the closest hospital in the rest of New York State (including Suffolk County- the eastern part of Long Island)

> 10 miles from one of the three included Connecticut hospitals

The Analysis

1. Find the top three closest hospitals for each block group (centroid)
   - QGIS origin-destination (O:D) matrix to estimate travel distance
     - Straight line analysis; does not take into account practical or geographic barriers
2. Map Emergency Care Deserts
   - Created radii (buffers) of the appropriate distance around each hospital (5, 10, or 15 miles)
3. Calculate the population in Emergency Care Deserts
   - Used 2020 Census block group population data (16,013 block groups with population in NYS)
   - Found intersection of the ECD radii with block group population layer
   - Calculated resulting area of space NOT covered by buffers (i.e. areas in ECD):

Population in ECD= (block group area within ECD/total block group area) * block group population

The Current Hospital Landscape

Summary Statistics: Distance from Block Groups to the Top 3 closest Hospitals

	Min. Distance (mi)	Max. Distance (mi)	Median (mi)
1st Closest Hospital	0.01	41.2	1.7
2nd Closest Hospital	0.02	42.8	3.2
3rd Closest Hospital	0.08	47.2	4.2

Median Distance Gaps Between the Closest hospitals for block groups, by area

	Median Difference between 1st and 2nd Closest hospitals	Median Difference between 1st and 3rd Closest hospitals
New York State	-0.93 miles	-1.94 miles
Downstate	-0.64 miles	-1.25 miles
Upstate	-2.32 miles	-5.87 miles

260 Hospitals with emergency departments (in and bordering NYS)

- 93 (36%) are downstate (Rockland & Westchester counties and below)
  - 58 of those are within the 5 boroughs of NYC
- 102 (39%) are in Upstate NY (north of Rockland and Westchester)
- 65 (25%) are in other states

Top 3 Closest Hospitals to Each Block Group

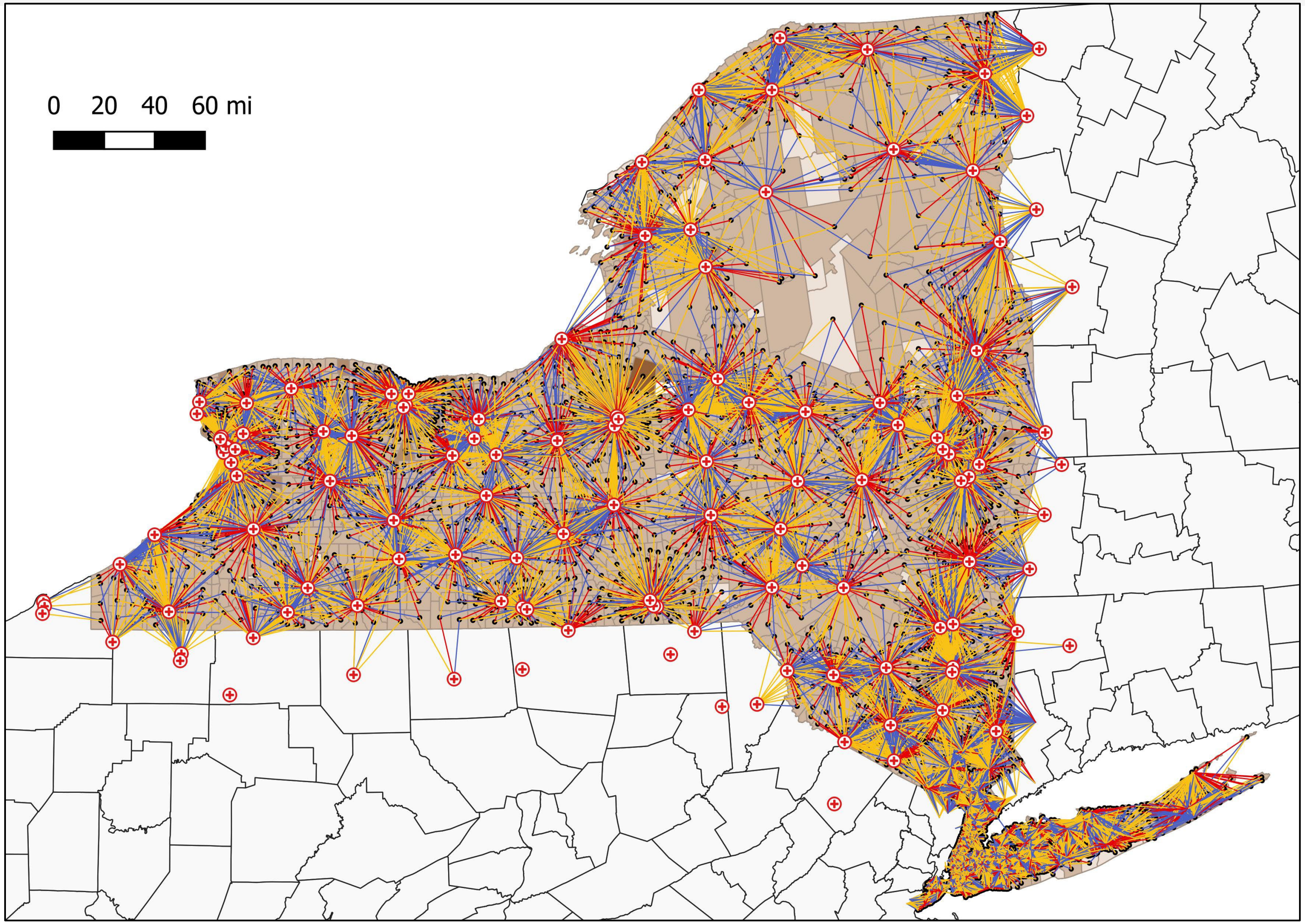
⊕ Hospital With Emergency Room

Distance from BG Centroid to:

- Closest Hospital
- 2nd Closest
- 3rd Closest
- • Block Group (BG) Centroids

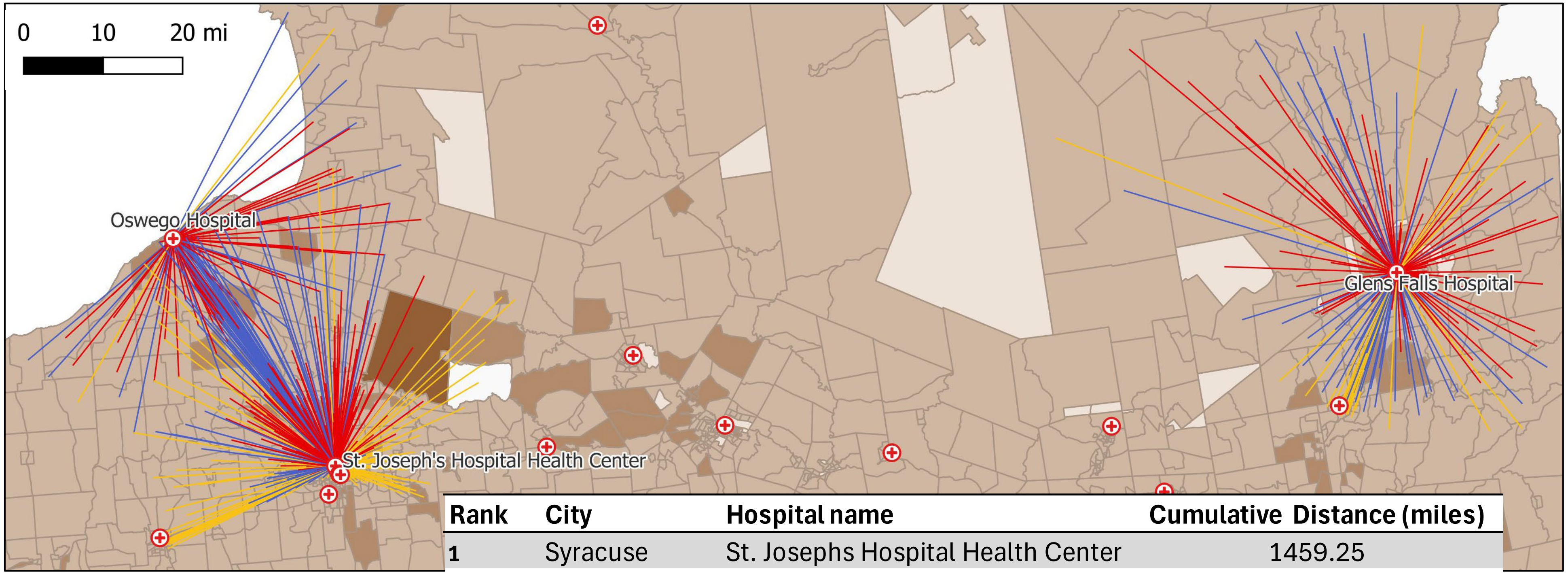
Total Population by Block Group

- < 500
- 500 - 2000
- 2000 - 3500
- 3500-5000
- > 5000
- County Boundaries



Top 3 Hospital "Hubs" (Upstate)

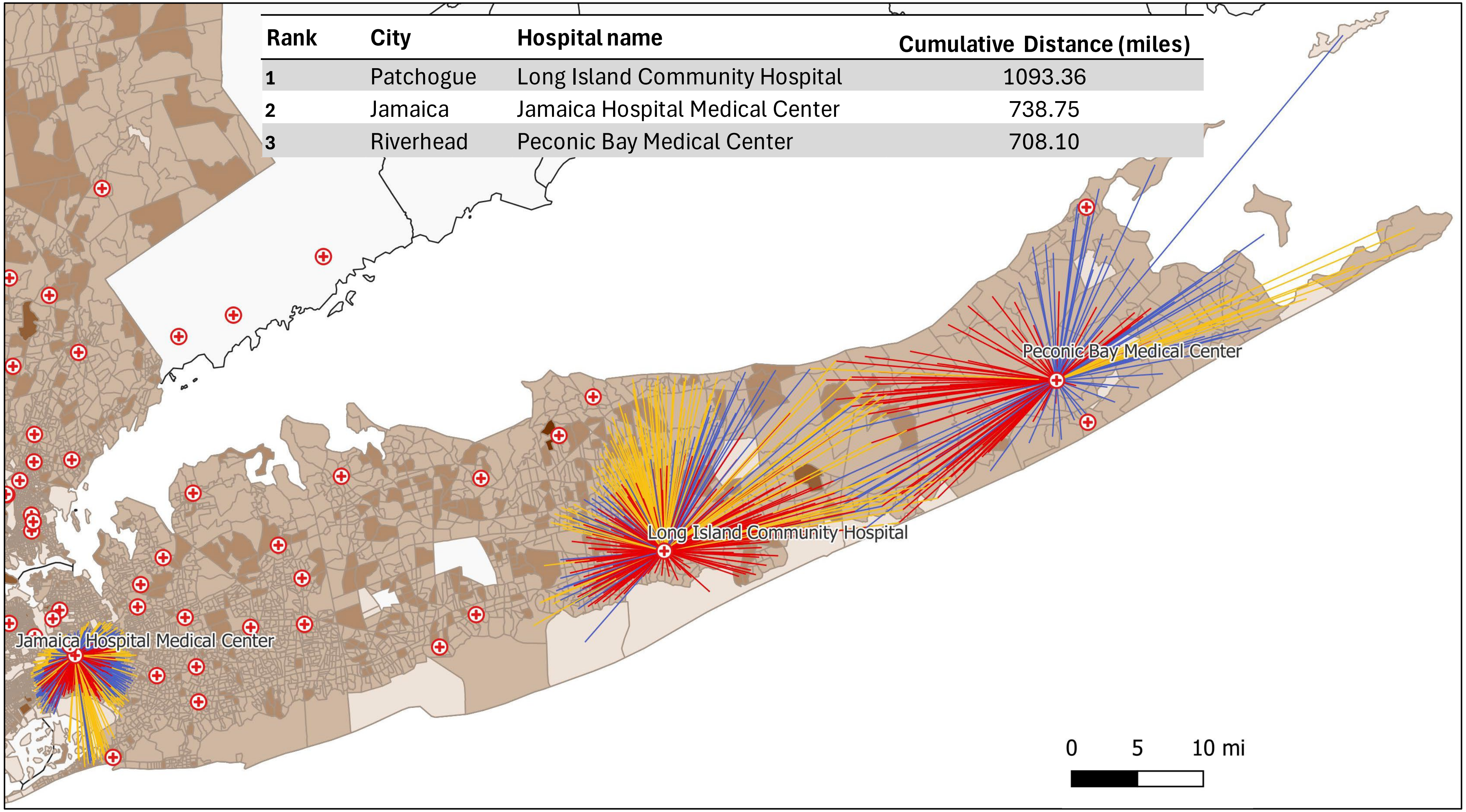
- Ranked hospitals by the cumulative distance of the red lines (distance to block group centroids that hospital is the closest option for)
- St. Joseph's Hospital Health Center in Syracuse had the most total distance covered to its closest block groups



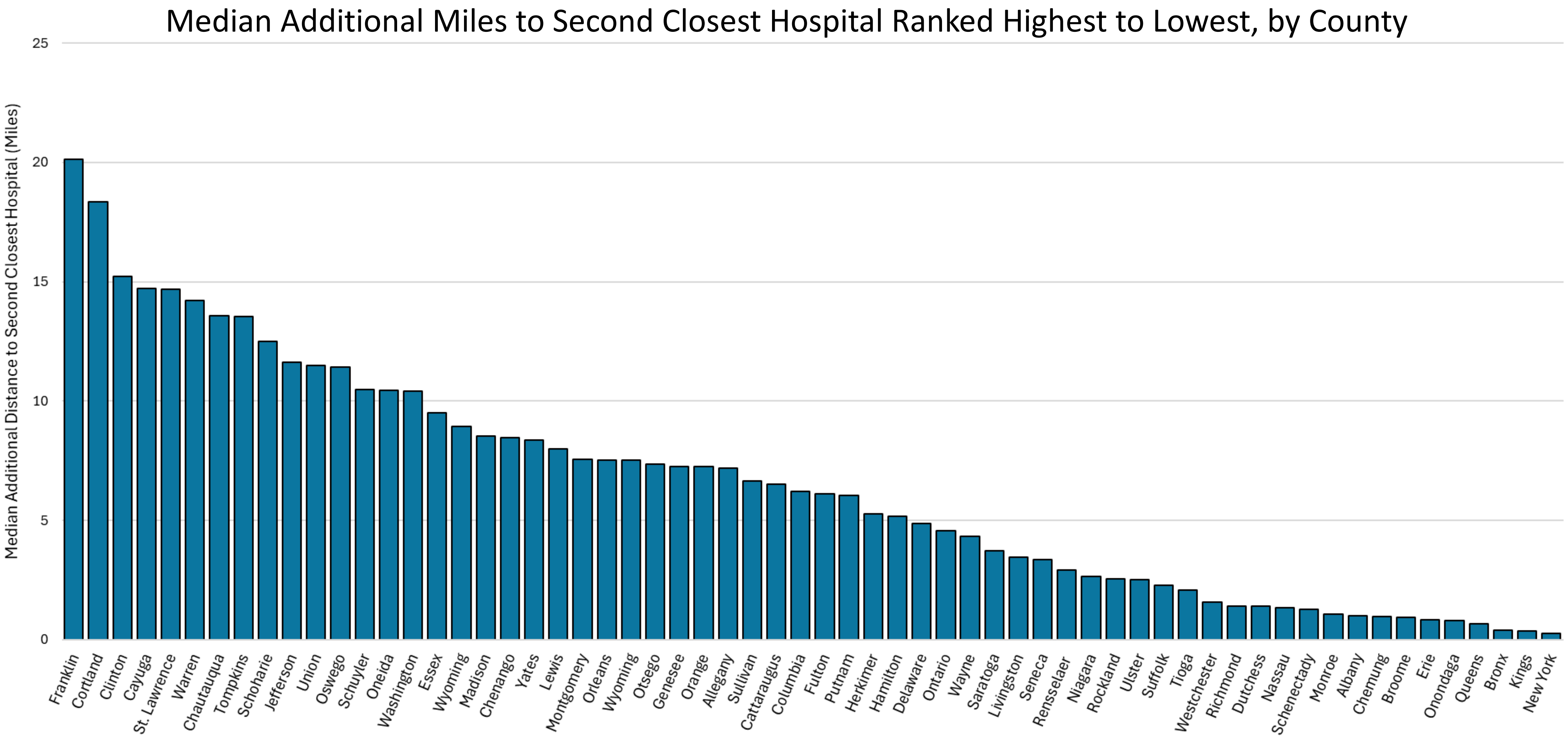
Rank	City	Hospital name	Cumulative Distance (miles)
1	Syracuse	St. Josephs Hospital Health Center	1459.25
2	Glens Falls	Glens Falls Hospital	968.26
3	Oswego	Oswego Hospital	816.66

Top 3 Hospital “Hubs” (Downstate)

Rank	City	Hospital name	Cumulative Distance (miles)
1	Patchogue	Long Island Community Hospital	1093.36
2	Jamaica	Jamaica Hospital Medical Center	738.75
3	Riverhead	Peconic Bay Medical Center	708.10



Most Geographically Vulnerable Counties



Most Geographically Vulnerable Hospitals

Hospital	County	Population Hospital is Closest for	Average* Additional Distance to Next Closest Hospital (miles)	"At risk" of closure?
Alice Hyde Medical Center	Franklin	33,696	18.7	Yes
Oswego Hospital	Oswego	98,116	16.7	**Yes**
Claxton Hepburn Medical Center	St. Lawrence	21,925	15.4	Yes
Clifton-Fine Hospital	St. Lawrence	17,158	14.8	**Yes**
Adirondack Medical Center	Franklin	25,055	14.4	No
Champlain Valley Physicians Hospital	Clinton	74,344	13.9	No
Guthrie Cortland Medical Center	Cortland	65,670	12.4	Yes
Auburn Memorial Hospital	Cayuga	80,788	12.1	**Yes**
Glens Falls Hospital	Warren	129,498	11.6	No
Cayuga Medical Center at Ithaca	Tompkins	106,308	11.4	No

Bolded = "most" at risk; over 25% net patient revenue (NPR) from Medicaid and government appropriations
*Weighted Average by block group population

New York State's Hospitals: Where is the Risk?

Hospitals With Emergency Departments in and Around New York State

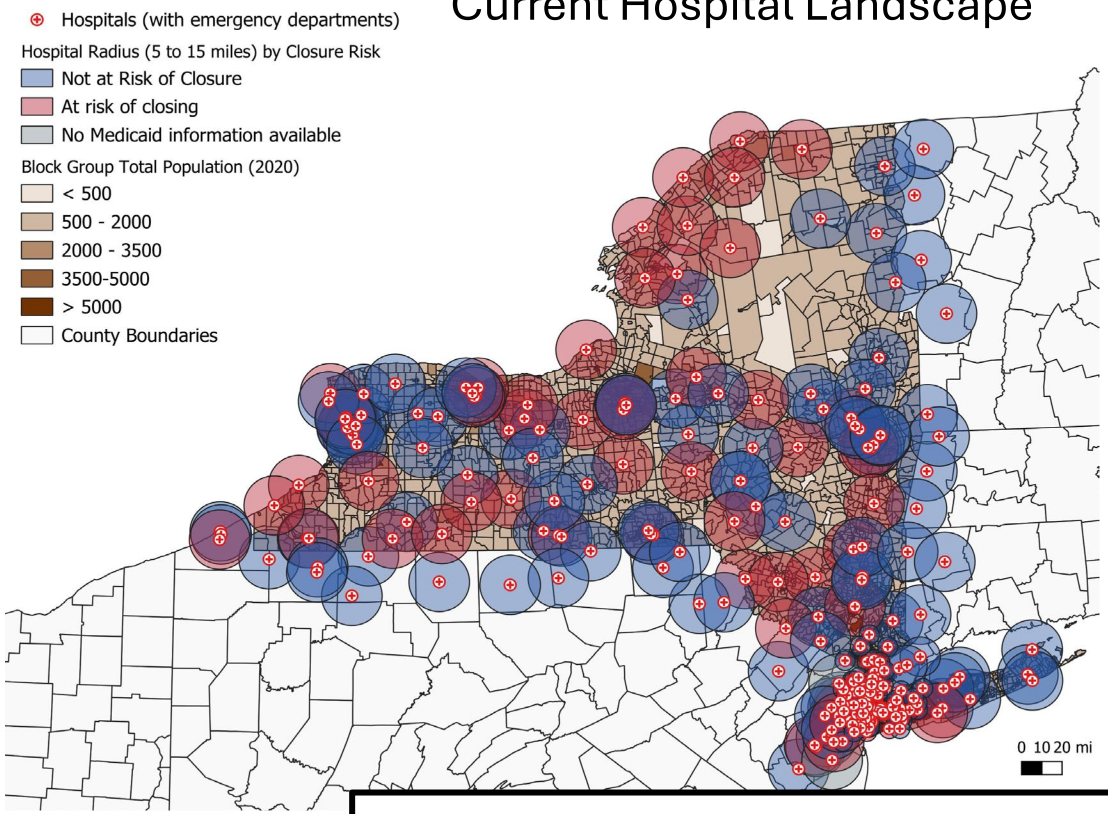
	All Areas	NYC+*	Upstate/Suffolk	Other States
Total Hospitals	260	85 (32.7%)	114 (43.8%)	61 (23.5%)
Between 20% and 25% NPR from Medicaid (Potential Risk)	32	2 (6.2%)	22 (68.8%)	8 (25.0%)
> 25% NPR from Medicaid (High Risk)	61	33 (54.1%)	28 (45.9%)	0

- 93 hospitals with emergency departments in New York State and surrounding areas (35.8%) are considered "at risk" under planned Medicaid cuts (>20% Net Patient Revenue from Medicaid and govt. appropriations)
- The majority of hospitals at "Potential Risk" are in Upstate/Suffolk (68.8%), but just over half (54.1%) of the "Most at risk" hospitals are in the NYC+ area

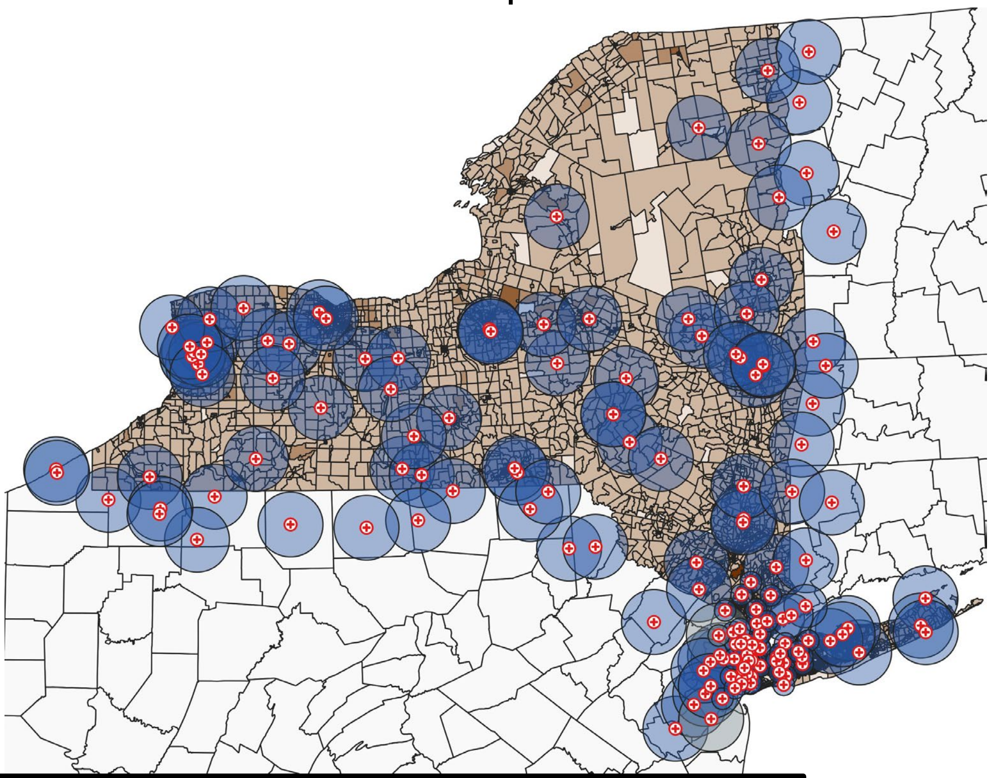
**Note: NYC+ includes the 5 boroughs of NYS and Nassau County, the western part of Long Island*

Visualizing the Impacts

Current Hospital Landscape



If All "At Risk" Hospitals Were to Close

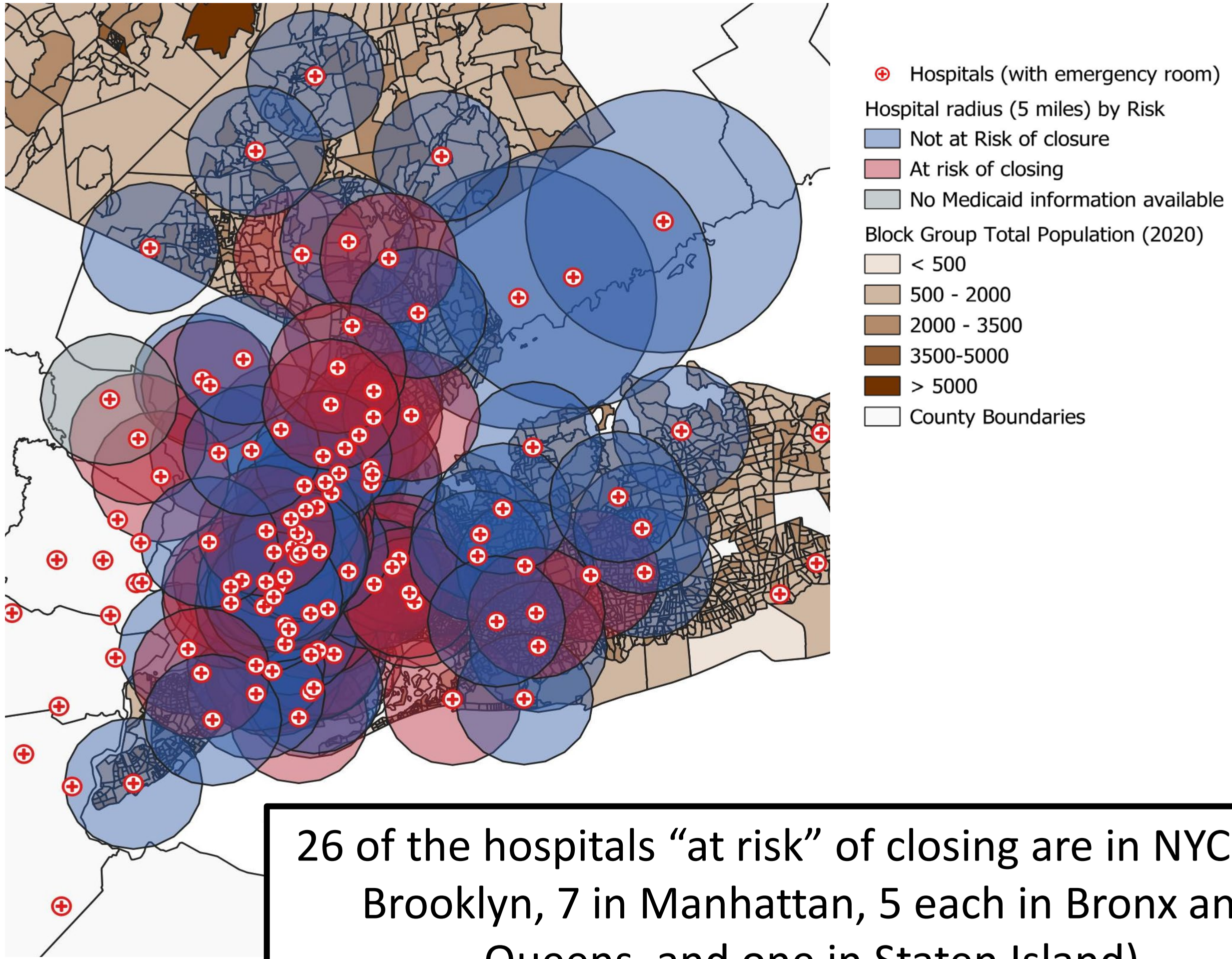


93 Hospitals are considered "at risk" of closure under this definition (Medicaid and Govt. Appropriations over 20% of their Net Patient Revenue)

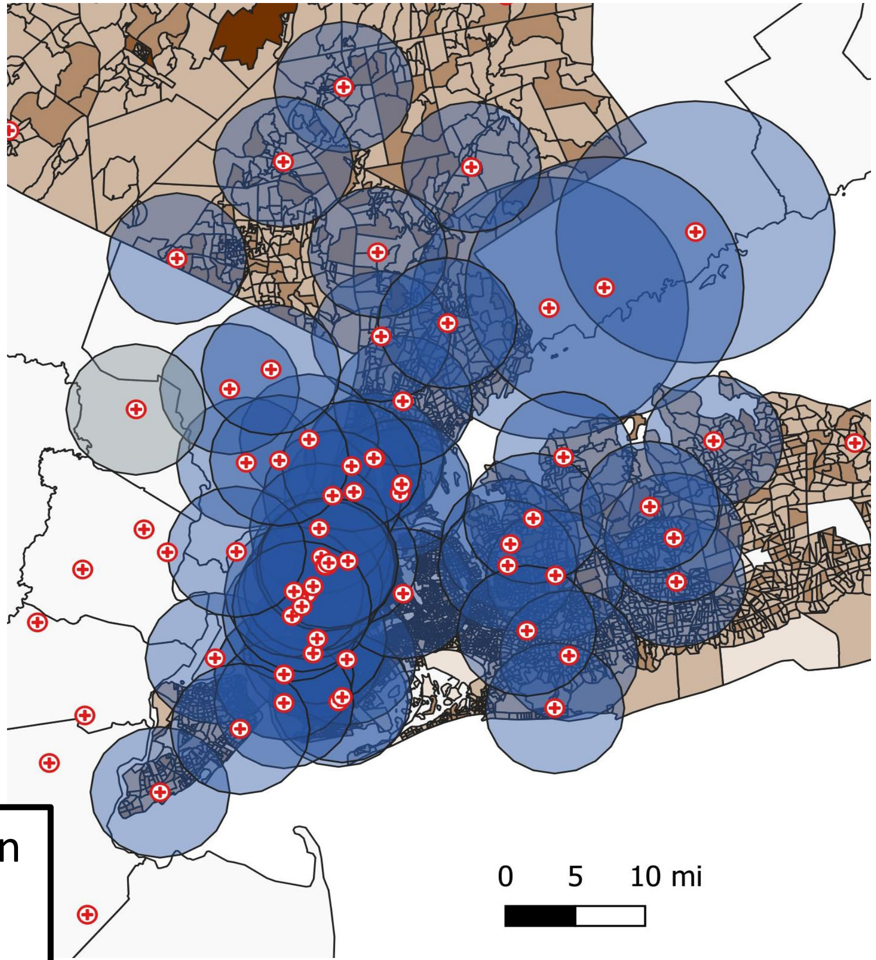
85 are in New York, 7 in New Jersey, and one in Pennsylvania

Visualizing the Impacts (cont.)

Current Hospital Landscape (NYC⁺)



If All "At Risk" Hospitals Were to Close (NYC⁺)

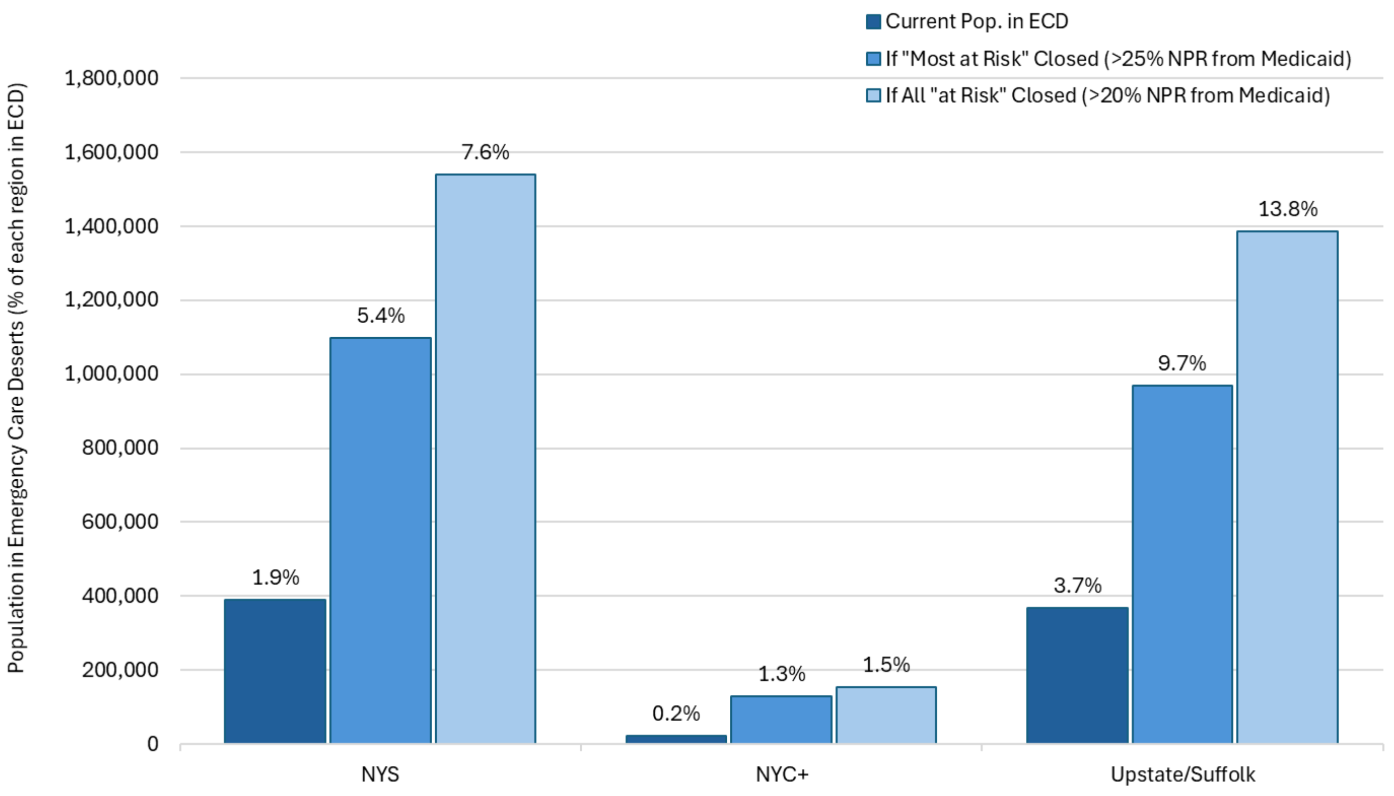


26 of the hospitals "at risk" of closing are in NYC (8 in Brooklyn, 7 in Manhattan, 5 each in Bronx and Queens, and one in Staten Island)

How Would ECD's Change?

- NYC+ is 50.5% of the state's population, but only 5.5% (21,416) of the state's Emergency Care Desert (ECD) population (389,455)
    - 95.5% of the ECD population (368,039) is in Upstate/Suffolk
- If the "most at risk" hospitals closed the ECD population in New York State would almost double (+182%; 708,633 people)
    - If all "at risk" hospitals closed, the impacted population would nearly triple from current levels (+295%; 1.15 million people)
- Emergency Care Deserts disproportionately impact Upstate/Suffolk but would also grow in NYC+ under closures.

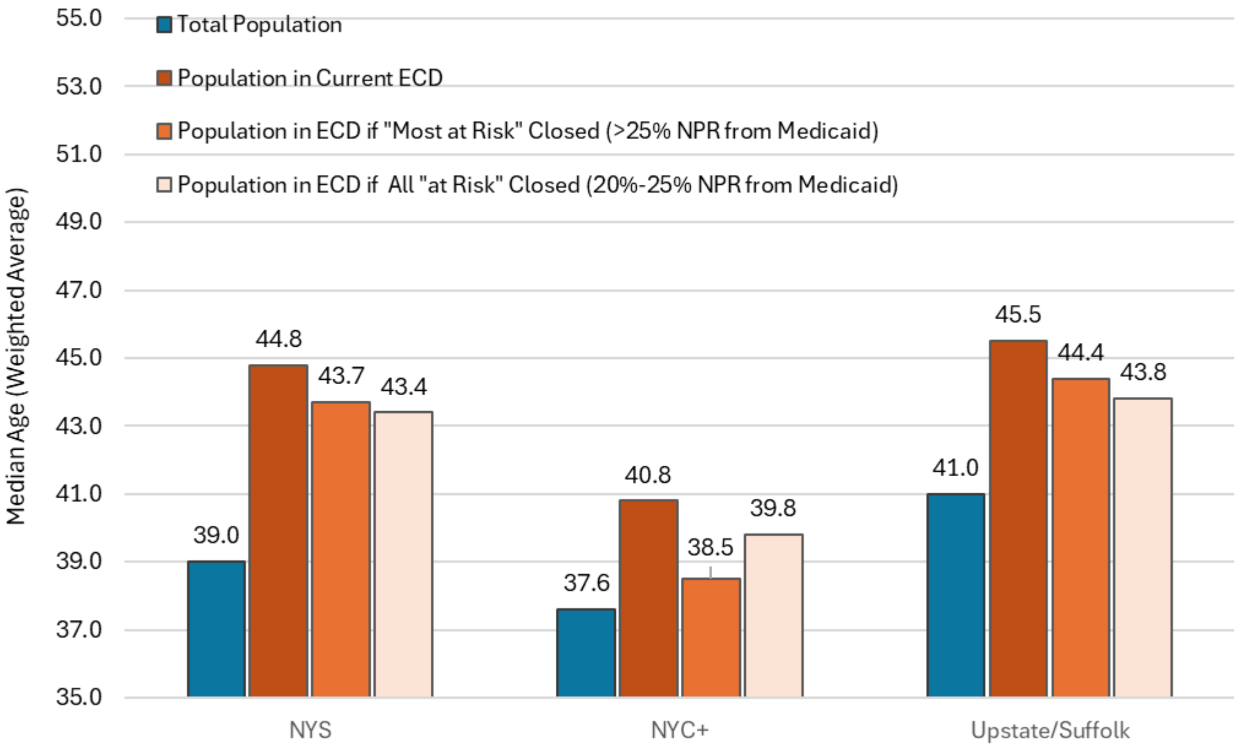
Share of the State and Each Region in Emergency Care Deserts, by Scenario



How Would ECD's Change? (cont.)

- Median age in NYC+ (37.6) skews younger than the rest of the state (41.0)
- The current population in ECDs (dark orange) tends to be older than the overall population in each region
- If the "most at risk" hospitals were to close, the age of the population impacted by ECDs would become younger
  - For the state overall and upstate/Suffolk, the average median age would become even lower if all "at risk" were to close
  - In NYC+ the ECD population if only the "most at risk" hospitals close skews younger than if all were to close

Median Age by Region and Emergency Care Desert Scenario



Conclusions

- Franklin, Cortland, and Clinton counties are the most geographically vulnerable to hospital closures
  - Population would have to travel over 15 additional miles on average to their next closest hospital if the first closest closed
- Half of the Top 10 most geographically isolated hospitals are in the North Country
- Upstate and Suffolk county are disproportionately impacted by emergency care deserts
- If all “at risk” hospitals closed, the population in Emergency Care Deserts would nearly triple
- Current population in emergency care deserts tends to be older than the population on average
  - Median age of care desert population would get younger if the “most at risk” hospitals were to close

References

Congressional Budget Office (CBO), August 2025. https://www.cbo.gov/publication/61367. Distributional effects of public law 119-21 |

Eisner, Emily, Kinnucan, Michael. 2025. “New York Hospitals will Close Under “One Big Beautiful Bill Act””. Fiscal Policy Institute. https://fiscalpolicy.org/wp-content/uploads/2025/07/2025.06.27-hospital-closures-FINAL.pdf

Jonk, Y., Milkowski, C., Croll, Z., & Pearson, K. 2023. Ambulance Deserts: Geographic Disparities in the Provision of Ambulance Services [Chartbook]. University of Southern Maine, Muskie School, Maine Rural Health Research Center.

Levinson, Zachary & Tricia Neuman. August 2025. “A Closer Look at the $50 Billion Rural Health Fund in the New Reconciliation Law:. Kaiser Family Foundation (KFF). https://www.kff.org/medicaid/a-closer-look-at-the-50-billion-rural-health-fund-in-the-new-reconciliation-law/

“New York State Health Care Reform Act (HCRA).” New York State Department of Health, 1 Mar. 2026, www.health.ny.gov/regulations/hcra/provider/provhosp.htm

Rao, Preethi, Lawrence Baker, Federico Girosi, Elaine Li, Rose Kerber, and Christine Eibner. February 2026. “State-Level Impacts of Key Medicaid Provisions in the One Big Beautiful Bill Act”. RAND Corporation, RR-A4098-1. As of April 13, 2026: https://www.rand.org/pubs/research_reports/RRA4098-1.html

Senator Edward J. Markey, Senator Jeffrey A. Merkley, Senator Ron Wyden & Senator Charles E. Schumer. June 2025. Letter to President Trump, Leader Thune, and Speaker Johnson on At Risk Rural Hospitals across the nation. https://www.markey.senate.gov/imo/media/doc/letter_on_rural_hospitals.pdf

Thank You!

Contact: lr523@cornell.edu